



DISCHARGE SUMMARY

Department of General Medicine

Patient Name	Rahul Patel	UHID	SMH-208841
Age / Sex	61 Years / Female	IP Number	IP-26-04417
Date of Admission	22/09/2026 10:40	Date of Discharge	27/09/2026
Consultant	Dr. Anil Kulkarni, MD	Ward / Bed	Medical Ward 3 / Bed 12

DIAGNOSES

Diagnosis	ICD-10	Status
Diagnosis: Type 2 diabetes mellitus with hyperglycaemia	E11.65	Principal, active
Diagnosis: Chronic kidney disease stage 3a	N18.31	Secondary, chronic
Diagnosis: Essential hypertension	I10	Secondary, chronic
Diagnosis: Hypothyroidism	E03.9	Secondary, on treatment
Diagnosis: Dyslipidaemia	E78.5	Secondary, on treatment

PRESENTING COMPLAINTS

61 year old woman, known diabetic for 12 years and hypertensive for 9 years, presented with 3 days of increased thirst, frequent urination, fatigue and giddiness. No chest pain, fever or vomiting. Home fasting sugars had been 220 to 280 mg/dL over the past week.

PAST HISTORY

Type 2 diabetes since 2014, hypertension since 2017 and hypothyroidism since 2019. Chronic kidney disease was noted on routine labs in 2025. No known drug allergies. Non-smoker, no alcohol.

EXAMINATION ON ADMISSION

BLOOD PRESSURE 148/92 mmHg	PULSE 84 / min	SpO2 98 % (RA)	TEMPERATURE 98.4 F	WEIGHT 68 kg
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Mild dehydration. Chest clear, heart sounds normal, abdomen soft. No pedal oedema.

ALLERGIES AND ALERTS

No known drug allergies. Renal impairment: avoid NSAIDs and review doses of renally cleared medicines. Glimepiride carries a risk of low sugars: counselled on symptoms and on carrying glucose.



HOSPITAL COURSE AND INVESTIGATIONS

Rahul Patel | UHID SMH-208841

HOSPITAL COURSE

Admitted with uncontrolled hyperglycaemia. Intravenous fluids and a basal-bolus insulin schedule were started and blood sugars settled to 140 to 180 mg/dL by day 3. Renal function was monitored daily; creatinine improved with hydration and was stable at discharge. Potassium was mildly raised on admission and was managed with diet. Urine routine showed no infection. Insulin was stopped on day 4 and oral agents were resumed.

LABORATORY INVESTIGATIONS

Sample collected: 22/09/2026 11:30 (on admission)

Test	Result	Flag	Unit	Reference
HbA1c	8.4	H	%	< 7.0
Fasting glucose	164	H	mg/dL	70 - 100
Creatinine	1.5	H	mg/dL	0.6 - 1.3
eGFR	47	L	mL/min/1.73 m2	> 60
Potassium	5.2	H	mmol/L	3.5 - 5.0
Sodium	136		mmol/L	135 - 145

Repeat sample collected: 27/09/2026 07:50 (before discharge)

Test	Result	Flag	Unit	Reference
Creatinine	1.4	H	mg/dL	0.6 - 1.3
Potassium	4.9		mmol/L	3.5 - 5.0

OTHER INVESTIGATIONS

- ECG: sinus rhythm at 82/min, no acute ST-T changes.
- Chest X-ray: normal cardiac size, clear lung fields.
- Ultrasound abdomen: both kidneys normal in size with mildly increased echogenicity.
- Urine routine: glucose 3+, no protein, no pus cells.



DISCHARGE MEDICATION AND FOLLOW UP

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MEDICINES ON DISCHARGE

#	Medicine	Strength	Dose and frequency	Remarks
1	Tab Metformin	500 mg	Twice daily	After meals
2	Tab Glimepiride	1 mg	Once daily	Before breakfast
3	Tab Telmisartan	40 mg	Once daily	Morning
4	Tab Amlodipine	5 mg	Once daily	Morning
5	Tab Atorvastatin	20 mg	Once daily	At night
6	Tab Levothyroxine	75 mcg	Once daily	Empty stomach
7	Tab Pantoprazole	40 mg	Once daily	Before breakfast

ADVICE ON DISCHARGE

- Diabetic diet with low salt and low potassium foods. Walk 30 minutes daily. Drink 2 litres of fluid a day.
- Check fasting and post-meal sugars at home and bring the record to review.
- Avoid painkillers of the NSAID group such as diclofenac and ibuprofen because of the kidney disease.
- Return at once for vomiting, breathlessness, very low sugars, reduced urine or swelling of the feet.

FOLLOW UP

Review in Medicine OPD in 1 week with HbA1c, creatinine, potassium and TSH reports. Nephrology opinion advised if creatinine rises above 1.8 mg/dL.

A. Kulkarni

Dr. Anil Kulkarni, MD (Medicine)

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